

MRI Lumbar Spine — Report

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Patient: Patient X (MRN: APX-2026-04421) · Date of scan: 2026-04-08 · Reporting radiologist: Dr. L. Chen, FRCR

Clinical Indication

58 F with 6-week history of progressive lower back pain radiating to the left leg, L5 dermatomal distribution, positive SLR. Rule out lumbar disc pathology.

Technique

Sagittal T1, T2, STIR; axial T2 from L1 to S1. No contrast administered.

Findings

Alignment: Lumbar lordosis preserved. No spondylolisthesis.

Vertebral bodies: Normal height and marrow signal throughout L1-S1. No fractures, no marrow infiltration, no enhancing lesion.

L4/L5 level: Posterolateral disc protrusion measuring 6mm extending into the left lateral recess, contacting and displacing the traversing left L5 nerve root. Mild facet joint arthropathy bilaterally. Ligamentum flavum thickened to 4mm. Moderate left foraminal narrowing. Right foramen patent.

L5/S1 level: Disc desiccation with 2mm broad-based bulge. No focal protrusion. Both neural foramina patent.

L3/L4 level: Disc desiccation only. No protrusion, no significant facet arthropathy. Central canal and foramina patent.

Upper levels (L1/L2, L2/L3): Unremarkable.

Conus medullaris: Terminates at L1, normal signal.

Paraspinal soft tissues: No mass, no abscess, no significant abnormality.

Impression

- Left posterolateral L4/L5 disc protrusion (6mm) compressing the traversing left L5 nerve root, with moderate left foraminal narrowing. Correlates with clinical L5 radiculopathy.
- Mild multi-level degenerative change. No central canal stenosis. No red-flag features (no mass, no infection, no fracture).

Recommendation

Findings consistent with mechanical L5 radiculopathy. Clinical correlation. Consider conservative management trial; if no improvement at 6 weeks or if neurological deficit progresses, escalate per pathway.

Report electronically signed by Dr. L. Chen, FRCR on 2026-04-08 17:42.